Lapatinib + capecitabine

National Cancer Control Programme (NCCP) 00217 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending

PRACTICAL TREATMENT GUIDE

Your practical tablet plan

Both anticancer medicines are taken by mouth. This regimen itself requires no infusion pump or port.

STEP 1

Before starting

Confirm dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) result, blood tests, cardiac assessment, medicines review and the two tablet schedules.

STEP 2

Collect and check the prescription

Confirm tablet strengths, doses, Days 1–14 for capecitabine and continuous lapatinib. Keep in the original packaging and out of reach of children.

STEP 3

Food and medicine timing

Take lapatinib at least one hour before or after food, consistently in the same relation to meals. Take capecitabine with food or within 30 minutes after eating. Swallow tablets whole. A pharmacist can help arrange the different food timings.

STEP 4

Before each new cycle

Keep blood and clinical-review appointments even though treatment is at home. Bring your medicine list and tell the team about missed or held doses.

Interactions

Check all prescription, over-the-counter and herbal medicines with the oncology pharmacist. Acid-reducing medicines can affect lapatinib absorption: NCCP specifies separation by at least two hours, while the product information cautions against acid-suppressing combinations. Do not start or stop a necessary acid medicine yourself; agree a plan. Avoid grapefruit and report any recent brivudine use.

Missed or vomited tablets

Do not make up or double a dose. Follow the written instructions and call if unsure.

If you already have a port

Ask the line team whether it is still needed and how it should be maintained or removed. Removal is a separate planned procedure.

Sources: NCCP 00217 v7 · official regimen · EMA Tyverb product information. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.